

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: An Observational Registry of Abatacept in Patients With Juvenile Idiopathic Arthritis **Short Title/Acronym:** Abatacept JIA Registry (PRINTO/PRCSG Registry) **Protocol Number:** IM101-240 **NCT Number:** NCT01357668 **Sponsor Name:** Bristol-Myers Squibb **Investigational Product:** Abatacept **Phase of Development:** Observational Study / Registry (Phase IV Equivalent) **Medical Monitor:** Bristol-Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To describe the long-term safety of abatacept for the treatment of juvenile idiopathic arthritis (JIA) in routine clinical practice by quantifying the incidence rates (IRs) of serious adverse events (SAEs) and events of special interest (serious infections, autoimmune disorders, and malignancies) over a 10-year follow-up period. **Secondary Objectives:** To evaluate long-term effectiveness using the clinical 10-joint Juvenile Arthritis Disease Activity Score (cJADAS10), assess temporary and permanent discontinuations due to adverse events, and evaluate immunogenicity (anti-abatacept antibodies and thyroid antibodies).

2.2 Background & Rationale The short-term safety of abatacept when used in juvenile idiopathic arthritis (JIA) has been previously demonstrated. However, long-term safety requires deeper evaluation. This international registry was established as part of regulatory requirements with long-term follow-up to fully characterize the safety profile of abatacept—especially rare events like opportunistic infections and malignancies—in routine, real-world clinical practice.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational, Longitudinal Registry **Control Type:** Single-arm (No specific control group, as-observed cohort) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: ~587 patients (based on interim analysis cohort size) **Number of Sites:** 70 clinical sites across 23 countries (in the PRINTO and PRCSG networks) **Estimated Study Start:** 30-Jan-2013 **Estimated Primary Completion:** 02-Jan-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Diagnosis of JIA (any subtype). 2. Age ≤ 18 years at the time of enrollment, unless currently or previously enrolled in an abatacept clinical trial and received abatacept. 3. Receiving abatacept at the time of enrollment as per the treating physician's decision or received abatacept in a clinical trial. 4. Parent or legally acceptable representative willing to participate in the study and sign the informed consent.

4.2 Exclusion Criteria 1. Pregnant or nursing female at the time of enrollment. 2. Prior malignancies if the patient has not been malignancy-free for at least 5 years. 3. Any serious acute or chronic medical condition other than JIA, including chronic infection, which would compromise the patient's ability to participate in the study. 4. Known poor compliance with clinic visits (based on physician judgment).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Dosed per standard of care and treating physician's clinical discretion. **Route of Administration:** Intravenous (IV) or Subcutaneous (SC). **Duration of Treatment:** Long-term observation for up to 10 years.

5.2 Concomitant & Prohibited Therapy Permitted: Standard concomitant therapies for JIA (e.g., conventional synthetic DMARDs, methotrexate) allowable under routine clinical practice. **Prohibited:** Use of other unapproved investigational agents that may confound the primary safety endpoints of the registry.

6. Study Timeline & Visit Schedule

Visit ID	Window	Key Activities/Procedures
Baseline	Day 0	Informed Consent, Medical History, Registry Enrollment
Follow-Up	Every 3 to 6 Months	Routine Clinical Visits, Effectiveness check (cJADAS10), Safety & Infection tracking
End of Study	Year 10	Final Safety Assessment, Exit Check, Abatacept status evaluation

(Note: Data is gathered opportunistically based on routine clinical appointments).

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence Rates (IRs) per 100 patient-years of follow-up for serious adverse events (SAEs) and events of special interest (ESIs) including serious infections, autoimmune disorders, and malignancies. **Measurement Method:**

Clinical observation, electronic health records review, and standardized registry reporting over 10 years.

7.2 Secondary & Exploratory Endpoints * Effectiveness measured by clinical 10-joint Juvenile Arthritis Disease Activity Score (cJADAS10) over 5 to 10 years. * Temporary and permanent discontinuations of abatacept due to AEs. * Incidence of immunogenicity markers, including thyroid antibodies and anti-abatacept antibodies.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring and physician reporting into the PRINTO/PRCSG registry databases. **Serious Adverse Events (SAEs):** Targeted reporting of specific events including EBV, CMV, papilloma virus, primary/reactivated herpes zoster, tuberculosis, and opportunistic infections. **Data Monitoring Committee (DMC):** Managed jointly by the Bristol-Myers Squibb safety board and network coordinators from PRINTO/PRCSG.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: As-observed analysis cohort containing all enrolled patients evaluated according to STROBE (Strengthening the Reporting of Observational Studies in Epidemiology) guidelines. **Sample Size Justification:** As an observational registry, sample size is guided by real-world usage to yield substantial patient-years (e.g., >1,200 patient-years) to accurately quantify the incidence of rare long-term safety events. **Handling of Missing Data:** Handled in accordance with STROBE parameters for missing longitudinal registry data.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), the Declaration of Helsinki, and observational reporting standards (STROBE). **Monitoring Plan:** Standardized protocol utilized across 70 clinical sites to assure uniformity in the reporting of SAEs and routine clinical outcomes. **Audit Trail:** Secure database entries curated by international pediatric rheumatology network registries (PRINTO and PRCSG).

11. Study Identifiers & Governance

Primary ID: IM101-240 **Registry Number:** NCT01357668 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** An Observational Registry of Abatacept in Patients With Juvenile Idiopathic

12. Clinical Framework (JIA Specific)

Primary Condition: Juvenile Idiopathic Arthritis (JIA) **Diagnosis Threshold:** Clinically confirmed diagnosis of JIA (any subtype) **Medical Methodology:** Routine Clinical Biologic Practice **Optimization Target:** Low disease activity or inactive disease sustained over time (measured via cJADAS10)

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for long-term pediatric rheumatology management. **Education Protocol (HCP):** Registry reporting guidelines and standardized definitions for targeted infections and disease activity scoring. **Education Protocol (Patient):** Standard clinical counseling regarding long-term biologic therapies and infection risks. **Quality Audit Mechanism:** Periodic safety signal tracking across site networks.

14. Observation & Follow-Up Schedule

Total Duration: 10 Years **Onsite Visit Cadence:** Guided by routine clinical care practice (typically 3-6 month intervals). **Remote Monitoring Frequency:** Continuous longitudinal registry data reporting. **Checklist Review Interval:** Annual database reviews.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				